

Lives of the Infamous: Identity Linkage Potential and Everyday Information Practices among People Living with HIV

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Abstract

Antiretroviral therapy has made HIV medically manageable, yet long-term treatment continually brings HIV-related information into medication routines, digital devices, institutional records, and social relationships. Prior research shows that disclosure is relational and sometimes involuntary, but less is known about how the possibility of identity linkage shapes everyday information practices before HIV status is actually recognized, or why information that people understand and trust may remain unacted upon. We conducted a qualitative field study in a large Chinese city, combining participant observation, 22 semi-structured interview sessions with 20 participants, including 16 people living with HIV, retrospective accounts, and exploratory online observation. Inductive thematic analysis identified three interconnected practices: regulating engagement with information, navigating identity-linked information flows, and translating information into everyday life. We develop identity-linkage potential as the situated possibility that a particular audience can access a cue and interpret it as evidence of a person's HIV status. This potential shaped participants' engagement with information, management of artifacts and records, access to support, and ways of acting on information. Within a situation, the same action could alter all three practices; across situations, its consequences could reshape later sources, audiences, channels, support resources, and everyday arrangements. Accurate and trusted information could therefore be acted on directly, discreetly, or through others, but could also be deferred or abandoned. Lower identity linkage sometimes protected ordinary life while limiting HIV-specific support. For digital library research, these findings highlight the need for privacy-sensitive access, intelligible information flows, low-visibility participation, and multiple viable routes from information to action.

CCS Concepts

• **Information systems** → **Digital libraries and archives**; • **Security and privacy** → **Social aspects of security and privacy**; •



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Keywords

information practices, identity-linkage potential, information use in everyday life, HIV, privacy-sensitive information access, peer knowledge infrastructures

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“...[T]hose billions of existences destined to pass away without a trace ... What snatched them from the darkness in which they could, perhaps should, have remained was the encounter with power ...”

Michel Foucault, “Lives of Infamous Men” [21, p. 161]

1 Introduction

Antiretroviral therapy (ART) has transformed HIV into a chronic disease that can be managed over decades. However, medical manageability has not made HIV-related information socially normal. Sustaining treatment requires daily medication, regular testing, clinical follow-up, and continued participation in digital and medical systems. These activities bring HIV-related information into everyday life through medication packaging, bodily routines, digital devices, interpersonal interactions, and institutional records. None of these objects or activities reveals HIV status by itself. This type of cue becomes consequential only when a particular audience can access it, interpret its meaning, and connect it to a particular person. Persistent stigma makes such possible connections socially consequential [9, 32, 47].

For example, a person with HIV can fully understand a basic piece of information about ART: that antiretroviral medication should be taken every day. However, this individual will need to make an additional effort to deal with the original packaging of the antiretroviral medication, as it may reveal their HIV status [42]. Thus, the medical information itself is neither rejected nor misunderstood. The difficulty arises when putting that information into practice creates cues that others might connect to HIV. This

individual's adherence, therefore, depends on more than applying medical knowledge. It also requires an additional information practice: changing how the medication is labeled, stored, and taken so that they can continue treatment without enabling unintended identity linkage.

Three established bodies of research connect to this phenomenon. Research on chronic illness information work has shown that people living with HIV (PLHIV) continually seek, interpret, evaluate, share, and use information to manage illness [24]. Information-practice research further situates such activities within bodily experience, material environments, social relations, and everyday routines [37, 52]. HIV disclosure and privacy studies have also shown that disclosure may be selective, gradual, or involuntary, and that medication, medical records, and digital platforms can expose HIV-related information to unintended audiences [10, 46, 60].

Together, these studies explain much of the information activity and disclosure risk surrounding long-term HIV care. They do not fully explain, however, what happens before HIV status actually becomes known to others: how the possibility that an ordinary action, object, or record could be connected to that status begins to reshape information practices in everyday life. Even when no one ultimately makes such an inference, the possibility of this connection may already have changed how medication is stored, where it is taken, which information channels remain usable, and what kinds of support can be sought.

To address this gap, we conducted an inductive study based on participant observation and semi-structured interviews involving 20 participants, including 16 PLHIV, in a large Chinese city. We observed and analyzed participants' everyday information practices, tracing how HIV-related information and cues entered everyday life and could be accessed, connected to HIV status, and circulated across social relationships, technological platforms, and institutional systems. Through this analysis, we theorize how ordinary information-related actions become consequential before anyone actually infers a person's HIV status, and how the audiences, cues, and possible identity linkages generated by those actions recursively shape subsequent information practices and life possibilities.

From this perspective, these information practices are not peripheral responses to HIV or merely individual strategies of privacy management. Together, they constitute an ongoing and recursive process through which HIV-related information and cues enter everyday life, become accessible and connectable to identity, circulate across social relationships and sociotechnical systems, and are translated into arrangements that make treatment and ordinary life sustainable. PLHIV therefore confront not only discrete decisions at moments of diagnosis or disclosure, but also a continuing informational burden that shapes their treatment, employment, family relationships, intimacy, and possible futures. In this sense, our study returns to Foucault's "lives of infamous men" — not to cast PLHIV as infamous, but to attend to ordinary lives made legible and potentially vulnerable through institutional encounters, informational records, and persistent stigma, as well as to the additional information practices required to sustain those lives. This burden should not be understood as an individual problem of privacy management, because it is produced by sociotechnical conditions that shape who can encounter HIV-related cues, how those cues can be associated with a particular person, and how information about that

person's HIV status can then circulate. Recognizing these conditions and understanding how they operate are therefore necessary first steps toward better supporting PLHIV in sustaining treatment and ordinary lives.

This study makes three contributions. First, it identifies how anticipated identity linkage can prevent accurate, trusted health information from becoming practicable, extending accounts of chronic-illness information work [24, 27]. Second, it explains how acting on information can reshape the conditions of subsequent practices, extending situated accounts of information practices [25, 37]. Third, it shifts attention from disclosure events to the audience-dependent recognizability of ordinary actions and records, showing how privacy protection and access to support can become interdependent [10, 45, 58]. For digital library and information-service research, these contributions direct attention beyond obtaining relevant information to the visibility of engagement and the feasibility of acting on it.

2 Literature Review

2.1 HIV Treatment, Stigma, and Health Information Behavior

ART has shifted research attention from acute mortality toward HIV treatment access and the everyday management of a chronic condition. Research on chronic illness shows that such management depends on continuous information behavior rather than the one-time acquisition of medical knowledge. For example, Hogan and Palmer suggest chronic-illness information work as the effort required to locate, sort, interpret, assimilate, and share information across an illness trajectory [24]. Subsequent research has examined how information becomes usable and is used in everyday life. First, information does not become useful automatically when available or understood. People must adapt medical advice to their routines and other resources at hand before they can act on it [25, 27]. Second, chronic-illness information behavior involves more than the application of formal medical knowledge. People learn through epistemic, social, and corporeal sources and use biomedical information to understand their bodies, navigate social relationships, and assert autonomy [37, 64]. Together, these studies show that health information behavior in chronic illness extends beyond acquiring and understanding information to its ongoing interpretation, sharing, and use in everyday life.

For PLHIV, HIV treatment also becomes part of their everyday lives. ART medication-taking is sustained through recurring times, places, meals, pillboxes, alarms, family reminders, and other elements of daily organization [26, 50, 51, 56]. HIV knowledge can also be obtained socially. PLHIV, family members, peers, and service providers seek, interpret, verify, and share information together, while clinics, community organizations, support groups, homes, media, and the Internet operate as different locations of learning [34, 58]. Digital communities further connect information with support and life reconstruction [7, 13]. These sociotechnical arrangements help PLHIV understand information and act on it, but they also place HIV-related information in these routines, objects, relationships, accounts, and records that other people may encounter.

For example, ART medication packaging may be recognizable, adherence routines may be observed, and institutional or platform records may alter who encounters sensitive information [33, 42, 60].

These potential encounters during these information practices matter because medical progress has not yet removed HIV-related moral judgment, social exclusion, or unequal access to support [28, 47, 67]. HIV treatment produces recurring information associated with HIV status that remains socially sensitive and consequential because of such social stigma. In China, the possible social consequences of recognition are shaped through family obligations, face (*mianzi*), local social networks, work arrangements, and institutional governance [19, 32, 65].

These consequences can extend far beyond the immediate encounter. A life-course perspective directs attention to how HIV-related stigma may redirect occupational, relational, and reproductive trajectories, with consequences that may also extend to families and partners through linked lives [16, 17]. The concept of biographical disruption captures the possible depth of these effects: illness can interrupt established trajectories and anticipated futures [5]. Later studies caution that such disruption is not uniform; illness may instead reinforce existing uncertainty or be incorporated into continuity [8, 18]. For PLHIV, this suggests that biographical disruption may arise not only from diagnosis or illness itself but also from the social consequences that follow when others recognize their HIV status.

Studies across these areas show that when HIV-related information becomes part of everyday life, stigma can make its use and circulation socially consequential. These studies have demonstrated both anticipated and experienced forms of HIV-related stigma, including disclosure concerns and the consequences of being recognized by others as living with HIV [1, 32]. However, research usually examines these concerns in relation to a specific practice or outcome, such as HIV-status disclosure [6], concealment [53], medication adherence [48], or access to information and support [57]. Less attention has been paid to the additional information practices that may arise from the possibility that HIV-related information in everyday life could be connected to a particular person's HIV status, even before recognition occurs.

2.2 HIV Disclosure and Privacy

For PLHIV, HIV-status disclosure occurs within sociotechnical arrangements that shape who receives status information, how it is communicated or recorded, and what consequences may follow. Making HIV status available within these arrangements can enable access to information, care, and support while also exposing a person to stigma or other social harms. Nondisclosure may reduce unwanted recognition, yet it can also limit who can participate in a person's information and help network [57, 58]. Because these consequences vary across relationships, technologies, and institutional settings, disclosure cannot be understood as a binary choice between openness and secrecy.

Such disclosure can instead be selective, relational, and iterative. People weigh their goals and anticipated responses, communicate through particular relationships, and revise later decisions in light of prior outcomes [31, 44]. Process-oriented research formalizes this dynamic by linking disclosure goals and anticipated responses

to the disclosure event, its individual and relational outcomes, and subsequent disclosure decisions [10]. Other studies show that disclosure varies according to the recipient, context, content, and purpose. It may be voluntary, occur inadvertently, or be initiated by others without the person's consent [14, 44]. Recent information-behavior research further conceptualizes self-disclosure as a transition associated with changing health-management and psychosocial needs [6].

However, sociotechnical arrangements can constrain individual control over HIV-status disclosure by making the communication and recording of status information a condition of access to essential services. For example, to receive treatment, a person's HIV status must be available to clinicians and recorded within health-care systems. Once recorded, who can access this information and how it circulates are shaped not only by the individual but also by professional roles, technical infrastructures, and organizational rules [46]. Privacy scholarship therefore broadens the analysis from individual disclosure decisions to the sociotechnical organization of information access and circulation.

Palen and Dourish conceptualize privacy as an ongoing process of boundary regulation under changing social and technological conditions [45]. Dourish and Anderson extend this perspective by showing that privacy is produced collectively through shared practices and norms rather than controlled by one individual [15]. Contextual integrity further explains why the same information flow may be appropriate in one setting but problematic in another, depending on the actors, purposes, and norms governing its transmission [43]. HIV-specific research similarly shows that privacy is accomplished in relation to particular people and places [39]. Platform design and institutional governance can further configure potential audiences, disclosure expectations, and access to HIV-related data [33, 60].

Although disclosure research examines anticipated responses and deliberation about disclosure, it generally remains organized around disclosure as an actual or prospective event—whether and how it occurs, why it is withheld, and what consequences may follow. Privacy research widens the analysis by showing that access to and circulation of HIV-related information are organized through relationships, technologies, and institutions. Within some arrangements, status information must be communicated and recorded to receive care [46]; within others, selectively sharing it can enable access to information, peer knowledge, and support [54, 58]. These insights suggest that sociotechnical arrangements can bring particular audiences into contact with HIV-related information, enabling them to possibly infer or recognize an individual's HIV status. What remains less understood is how this possibility shapes the everyday information practices of PLHIV before recognition occurs, even when deliberate disclosure is neither planned nor underway.

2.3 Information Practices and Life Transitions

Models of information behavior often begin with an individual's information need and examine how that person seeks, uses, or transfers information, while taking psychological, interpersonal, environmental, and source-related factors as conditions in this process [62, 63]. In the context of everyday life, however, seeking, receiving, sharing, avoiding, interpreting, and using information

are not only individual responses to information needs; they are socially organized and situated accomplishments constituted through social relationships, shared norms, bodily experience, material settings, and institutional arrangements [41, 52, 55]. The concept of information practice has therefore been proposed as a means of understanding the social, embodied, material, and situated organization of information activity [35, 36, 38]. This perspective is particularly relevant in the context of chronic illness, in which people orient themselves through epistemic, social, and corporeal sources rather than relying solely on formal medical knowledge [37].

In everyday life with chronic illness, how people engage with health information is shaped by social relationships, daily routines, symptoms and other physical experiences, and material environments. These circumstances, for example, influence whether, when, and how people seek or avoid information. Both information seeking and avoidance may help people manage uncertainty. At the same time, affect, existing knowledge, social relationships, and the timing and form of information may intensify, narrow, suspend, or redirect their engagement with information [2, 11]. A person may seek information from some sources while avoiding others or particular types of content, and avoidance may moderate interaction with information rather than end it altogether [23]. These patterns demonstrate how everyday settings shape the information people encounter, the people from whom they receive it, and the extent to which they engage with it.

Changes in people's roles, relationships, or routines may reshape their information practices. Research on life transitions examines how people use information to make sense of and respond to such changes. For example, Willson shows how liminality changes information needs and how people compare their experiences with those of others to understand unfamiliar situations, while Ruthven distinguishes the processes of understanding changed circumstances, negotiating possible responses, and reaching a workable orientation [49, 61]. For PLHIV, diagnosis may mark a life transition, whereas disclosure may unfold over time as health-management and psychosocial needs change rather than occur as a discrete event [6]. During these transitions, PLHIV may reconsider their health, relationships, and future, while also changing how they seek, share, and use information. These transitional information practices can help them make sense of their changing circumstances [66].

Information practices can, in turn, change the social settings in which they occur. Within couples and families, seeking, sharing, and withholding health information can be considerate acts, but these practices can also produce surveillance, conflict, and attempts to control health-related behavior [3, 40]. Longitudinal research further illustrates that chronic illness can reorganize family information networks as health conditions, relationships, and roles change [4]. By changing who participates in information exchange, the roles participants assume, and the information they provide or withhold, these practices may alter care relationships, information pathways, and sources of support.

Thus, existing research shows that social settings shape information practices and that these practices can reshape relationships and information networks. However, the mechanisms underlying this reciprocal relationship remain insufficiently understood. Research on PLHIV has examined the information practices involved

in obtaining support, maintaining privacy, disclosing status, and managing treatment [6, 39, 48, 58], but it has not fully explained how changes in one everyday information practice affect other practices. Research on life transitions has examined changes surrounding diagnosis and disclosure, but less is known about how such transitions develop through the reciprocal reshaping of information practices and social settings in everyday life.

3 Method

3.1 Research Design and Setting

This study used an inductive design combining participant observation, semi-structured interviews, exploratory non-participant online observation, and retrospective life-course accounts. The study received institutional ethics approval from the participating hospital before fieldwork began. Because the approving institution was also the field site, its name and approval number are withheld for confidentiality and can be provided to the conference if required. Fieldwork took place during February and March 2026 at a specialist infectious-disease hospital and an affiliated peer-support organization in a large Chinese city. Researchers entered the setting as volunteers, observing routine organizational work, telephone interactions, counseling, ward-based volunteering, outpatient and inpatient encounters, and a community event, with 5 visits comprising approximately 60 hours of observation. More than 30 pages (over 30,000 Chinese characters) of fieldnotes were written immediately after each visit to record interactions, context, and researcher reflections.

The design provides both cross-sectional and retrospective evidence, and the data sources served complementary purposes. Interviews provided the primary evidence and were formally coded, including accounts of information practices in homes, workplaces, intimate relationships, and other settings that the researchers could not directly observe. Offline participant observation helped the researchers understand the clinical and peer-support settings in which some of these accounts were situated and provided contextual points of comparison with the interview narratives. Exploratory online observation supplied preliminary contextual knowledge and informed revisions to the interview protocol. Retrospective accounts within the interviews were used to reconstruct participant-described sequences and perceived connections between earlier and later situations.

Participants were recruited through the hospital, the peer-support organization, and a public online recruitment post. PLHIV participants were eligible if they were adults, had received a clinical HIV diagnosis, were receiving antiretroviral therapy, were clinically stable, and were able to provide informed consent and participate in an interview in Chinese. Hospital-based participants were screened by the peer-support organization and clinical staff, whereas online participants self-reported their diagnosis and treatment status.

Recruitment and participation were voluntary with compensation. The final sample comprised 20 participants: 16 PLHIV (including one peer volunteer), two family members, and two medical or community-organization staff members. As summarized in Table 1, the sample varied in age, gender, time since diagnosis, and relationship to HIV care and support. This variation broadened the range of circumstances available for cross-case comparison, with

these characteristics serving as descriptive context for interpreting participants' information practices.

3.2 Data Collection

We conducted 22 semi-structured interview sessions involving 20 participants. Five participants completed preliminary interviews online through audio calls on a meeting platform; the remaining sessions were conducted in person. Frank (B1) and Grace (B2) were interviewed again on the following day after a review of their initial interview records identified opportunities to explore information disclosure in intimate relationships in greater depth. Because formal analysis had not yet begun, the follow-up material was incorporated into their respective case records and coded alongside the other interviews. All interviews lasted more than 30 minutes, generally between 30 and 60 minutes, and followed the same core semi-structured protocol. The in-person protocol additionally included questions about hospitalization, participants' feelings about their hospitalization, and everyday life in the hospital. Interviewers used the protocol flexibly and invited participants to narrate their experiences in their own terms. All authors participated in data collection.

Participants received information about the study, data use, privacy protections, and withdrawal before providing informed consent. With permission, interviews were audio-recorded. Declining or withdrawing did not affect access to treatment, volunteering opportunities, or support services. Names, locations, institutions, and potentially identifying details were removed or generalized, and audio files and transcripts were restricted to the research team. The sessions produced more than 160 pages of Chinese-language transcripts, comprising over 200,000 words. When participants described changes over time, interviewers asked what preceded the change, what occurred, and what participants did afterward. These questions supported the construction of retrospective accounts.

Before offline fieldwork, we also conducted exploratory non-participant online observation [12, 29]. Using the Chinese term for "AIDS" as a search keyword, we reviewed several highly engaged public posts on Xiaohongshu (RedNote) and were admitted to one peer group established by a post author. From February to April 2026, the researchers observed naturally occurring group conversations without disclosing their research roles, posting, or interacting. The observation drew attention to intimate relationships, HIV-status disclosure, and online peer support and informed revisions to the interview protocol. The online observation itself was not included in the formally coded corpus, and no direct or searchable language from the posts or group conversations is reported in the findings.

All online and in-person interviews entered the same formal analytic corpus and were analyzed using the same procedures. Interview mode was retained as contextual information but did not determine the analytic weight given to an account. Offline fieldnotes were consulted during interpretation to situate participants' accounts within hospital and peer-support settings and to compare the reported arrangements with those observed in the field.

3.3 Data Analysis

We conducted inductive thematic analysis of the interview corpus, combining within-case temporal bracketing with a cross-case framework matrix [20, 22, 30]. First-cycle semantic coding identified information-related actions, objects, sources, audiences, risks, consequences, support, and everyday arrangements. Iterative within- and cross-case comparison produced three analytically distinct but empirically overlapping domains: regulating engagement with information, navigating identity-linked information flows, and translating information into everyday life. Research on information practices, disclosure and privacy, and chronic-illness information behavior served as sensitizing concepts during later interpretation and theme refinement.

We then examined situations involving possible identity linkage. A linkage episode required (1) a cue, such as an information item, object, record, or action; (2) an access route through which a particular audience could encounter it; and (3) empirical grounds for possible association with the participant and HIV. Association had to be supported by participants' concerns, audience responses, or contextual evidence rather than analyst inference alone. We separately coded audience knowledge, linkage status, participant responses, consequences, and alternative routes, retaining negative cases in which cues were not linked to HIV, linkage did not alter practice, recognition enabled support, or changes had other explanations.

A case-by-theme-by-time matrix compared arrangements before a reported change, associated events or feedback, and subsequent practices. Because most participants were interviewed once, it supported retrospective temporal bracketing rather than longitudinal causal inference. We distinguished connections within and across situations. Within-situation interweaving required evidence that one practice affected another in the same situation. Co-occurrence or double coding alone was insufficient. Across-situation connections required a participant-described link or a concrete change in an audience, relationship, routine, object, record, support arrangement, or institutional pathway. Temporal succession alone was insufficient. We recorded supporting evidence and plausible alternative explanations and therefore interpret these sequences as participant-described connections.

Table 2 summarizes this progression from first-order semantic codes to second-order themes and aggregate analytic categories, as well as the criteria used to distinguish identity-linkage episodes from competing explanations.

Across these analytic stages, coding was conducted from the Chinese originals. Quotations were selected and translated for the manuscript after theme development. Participant accounts form the primary evidence for experiential claims, while staff and volunteer statements are identified as professional or institutional observations and are not used as substitutes for PLHIV accounts.

4 Findings

Participants described a recurring tension between medical manageability and continued informational demands. ART and improving clinical indicators made HIV controllable. For example, Alice's undetectable viral load and rising CD4 count changed how she understood the condition, while Emma described her improving

Table 1: Characteristics of Interview Participants

ID	Pseudonym	Gender	Age	Time Since Diagnosis	Interview Setting
<i>People living with HIV</i>					
A1	Alice	Female	28	2 years	Online interview
A2	Bella	Female	25	1 year 8 months	Online interview
A3	Chloe	Female	28	2 years	Online interview
A4	Diana	Female	33	2 years	Online interview
A5	Emma	Female	21	1 year	Online interview
B1	Frank	Male	45	2 years 3 months	Hospital ward
B2	Grace	Female	44	1 year	Hospital ward
B3	Henry	Male	31	9 years	Hospital ward
B4	Ian	Male	37	Not reported	Hospital ward
B5	Jane	Female	55	5 years	Community support event
B6	Kate	Female	55	9 years	Community support event
B7	Leo	Male	29	1 week	Hospital ward
B8	Mark	Male	42	6 years	Hospital ward
B9	Nora	Female	55	10 years	Hospital ward
B10	Owen	Male	Approx. 40	Not reported	Hospital ward
<i>Family members</i>					
C1	Leo's mother	Female	Approx. 50	N/A	Hospital ward
C2	Paul	Male	21	N/A	Hospital ward
<i>Peer volunteer and staff members</i>					
D1	Nurse Mary	Female	50+	N/A	Nursing station
D2	Richard	Male	67	26 years	Peer-support organization
D3	Susan	Female	40+	N/A	Peer-support organization

Note. All names are pseudonyms. D2 is both a person living with HIV and a peer volunteer.

indicators as providing “a sense of assurance” (A5). At the same time, treatment kept HIV-related information present through medication routines, follow-up care, and decisions about who could know. As Alice put it, “We may only need to take medication every day, but we also have to keep a secret every day. We have not harmed anyone; we are simply living very carefully” (A1). Medical manageability therefore did not remove HIV-related information from everyday life. Instead, everyday treatment became one of the main settings in which HIV information had to be interpreted and handled.

We organize the findings around three interconnected information practices. *Regulating engagement with information* concerns how participants adjusted their exposure to information and decided which sources to trust. *Navigating identity-linked information flows* concerns how HIV-related cues could become accessible and meaningful to particular audiences. *Translating information into everyday life* concerns whether and how participants acted on clinical knowledge, institutional rules, and peer experience and incorporated them into everyday routines and arrangements.

The following subsections first examine each practice separately and then show how they interwove. Identity-linkage potential explained many connections among them, particularly when treatment, devices, records, or relationships made HIV status inferable. It was not, however, a general explanation for every change. Clinical feedback, emotional burden, competing demands, and reduced information need also shaped participants’ practices.

4.1 Regulating Engagement with Information

Regulating engagement involved more than seeking information or avoiding it. Participants adjusted their exposure as their needs and confidence changed. After diagnosis, Chloe searched repeatedly: “I first searched indiscriminately online. The more frightened I became, the more I searched, and the more I searched, the more frightened I became” (A3). She later passed over alarming posts while continuing to consult information from public-health agencies and major hospitals. Alice similarly narrowed her attention toward authoritative sources after her treatment results made medical explanations credible. These accounts describe movement from intense and emotionally difficult searching toward more selective engagement, rather than a complete withdrawal from HIV-related information.

Participation in peer environments was also selective. Bella sought help from the group when she faced a specific problem, but said that “most of the time I remain a lurker in the peer group” (A2). Lurking allowed her to retain access to peer experience without making her participation consistently visible. Her engagement therefore changed with need and comfort rather than following a stable preference for either participation or withdrawal.

Low engagement also had explanations unrelated to identity linkage. Kate rarely read public-account posts because work and household responsibilities occupied her attention, although she listened when the peer group shared a physician’s lecture (B6). Richard continued to encounter negative posts but said that they no longer affected him emotionally (D2). These accounts show

Table 2: Coding Structure and Analytic Criteria

Aggregate Category	Second-Order Themes	Illustrative First-Order Codes	Analytic Criteria
<i>Contextual codes</i>			
Persistent everyday embedding	Materialization and rhythmic organization of treatment; bodily and clinical feedback	Daily medication; reminders; concealed medication; recurrent tests; hospital contact; clinical indicators affecting trust and action	HIV-related information recurring in temporal, bodily, material, or institutional arrangements
<i>Information practices</i>			
Regulating engagement with information	Modulating intensity, timing, sources, trust, and participation visibility; withdrawal and re-entry	Extensive searching; swiping away content; consulting authoritative sources; lurking; asking when needed; leaving and later rejoining groups	Adjusting information exposure, timing, trusted sources, or participation visibility
Navigating identity-linked information flows	Reconfiguring artifacts and accounts; selectively expanding co-knowing networks; navigating technically and institutionally constrained control	Repackaging medication; discarding packaging; using alternative explanations; selective disclosure; leaving groups on shared devices; relocating care	Intervening in or adapting to pathways through which HIV-related information was accessed, interpreted, or circulated
Translating information into everyday life	Acting directly or discreetly; acting through another person or an alternative route; delaying, foregoing, or acting through trusted relationships	Establishing medication routines; covert dosing; delegating procedures; postponing treatment; declining benefits; family co-care; peer volunteering	Whether/how known or trusted information was enacted and incorporated into routines under bodily, material, relational, economic, temporal, and institutional conditions
<i>Cross-practice mechanisms and boundary conditions</i>			
Identity-linkage potential	Cue accessibility and inferability; perceived and realized linkage; anticipated or experienced consequences; alternative routes	Shared phones; overheard hospital calls; visible packaging; record-access concerns; hospital names; disclosure; support, exclusion, or institutional action	Required a cue/action, access by a particular audience, and empirically supported possible HIV inference; consequences and alternative routes coded separately
Interweaving of practices	Within-situation overlap; domain differentiation; changes in information roles	Repackaging limiting cues while sustaining adherence; shared devices shaping group access/audience boundaries; selective disclosure enabling support but not intimacy; recipient-to-peer information role	Same action or arrangement contributed to two or more practices; co-occurrence alone was insufficient
Connections across situations and boundary conditions	Consequences changing later information engagement, identity-linked information flows, ways of acting, or everyday arrangements; beneficial identity linkage; non-linkage explanations	Authority filtering after clinical improvement; group re-entry after shared-device arrangements changed; reduced browsing after life stabilization; low engagement associated with work demands or informational sufficiency	Earlier consequences changed later sources, audience/channel boundaries, support, action, or everyday arrangements; negative cases and rival explanations retained

that limited engagement may reflect competing demands, reduced need, or resilience. Regulation was therefore a situated adjustment between available information and a participant's current circumstances.

4.2 Navigating Identity-Linked Information Flows

Participants could redirect some pathways through which HIV information traveled, but they could not fully control them. Alice moved ART into a vitamin C container and used the container to support a non-HIV explanation when others asked about the pills (A1). She also changed where and when she took medication and described follow-up visits as ordinary errands. The medication would not disclose HIV by itself. Its linkage potential depended on whether another person could see it, recognize its significance, and accept or reject the alternative explanation.

Changes in audience access could alter the same information environment. Jane explained, "For a while, I left the group ... sometimes my child would use my phone. Now that my child hardly uses it, I have rejoined the group" (B5). The peer group had not changed, but the possibility that her child might encounter its traces had. Documents were similarly contingent. Nora recalled that before retirement, "The doctors could not mention this illness in the medical records or on the receipts. If they wrote it down, it would cause trouble." After retirement those documents became less consequential, "Now that I am retired, it no longer matters. But back then, all of these documents were extremely important. The leave slips, medical records, and so on could not mention the illness at all. It was terrifying." (B9). A record's linkage potential therefore changed with its administrative audience and purpose.

Institutional flows were harder for participants to assess or redirect. Some participants feared that records could be retrieved by authorities through an identity number at will or that a hospital

name spoken during a call could be overheard. Nurse Mary offered a narrower institutional account: electronic records were available only to authorized staff who deliberately retrieved them, rather than to any hospital worker (D1). Her observation cautions against treating record existence as automatic exposure. Yet uncertainty about permissions still affected participants because they could not readily determine who had access. Perceived linkage and technically possible access were therefore distinct, even when both shaped information practices.

Navigation could also involve allowing trusted people to become informed. Bella initially intended to tell no one, but later informed a small number of relatives and friends when treatment and emotional pressure became difficult to manage alone. “The most tangible support was the emotional understanding they offered in everyday life; they paid closer attention to what was happening to me.” (A2). Selective linkage preserved boundaries around other relationships while opening access to HIV-specific support. Lower linkage could protect continuity in some settings, but complete concealment could also exclude potential supporters.

4.3 Translating Information into Everyday Life

Information became consequential when participants tried to incorporate it into daily arrangements. For Emma, improving indicators strengthened her confidence in treatment and made reproduction imaginable after she learned about prevention of mother-to-child transmission. She described herself as “an ordinary person who needs medication” and said “I am no different from anyone else.” (A5). In her case, clinical evidence and future aspirations supported a treatment routine that could be sustained without HIV defining every part of life.

Understanding and accepting information did not always mean that participants felt able to act on it in their social lives. Bella largely believed the scientific conclusion of U=U (undetectable = untransmittable) but retained reservations about intimacy. Her imagined future shifted from wanting “a loving partner” toward preferring to live alone (A2). The difficulty here lay in the possible and complicated consequences of pursuing an intimate relationship. A relationship would require her to address disclosure, possible rejection, and responsibility toward a partner. Medically possible intimacy therefore remained difficult to sustain as a social future.

Institutional information could encounter a similar gap. Nora knew that legal action might produce compensation after an alleged institutional harm, but rejected that solution: “How could I sue? At most I would get some money, but the news would certainly spread ... How could I live with people pointing fingers at me?” (B9). A trusted contact later completed a benefit procedure on her behalf, yet she still did not feel able to use the money. Delegation reduced her visibility during the application without resolving the consequences she associated with the benefit. The alternative route relocated the problem rather than making the benefit something she felt able to use in everyday life.

Selective linkage could instead expand what information enabled. Richard connected the care he had received to his later peer-volunteer role: “I received treatment and care ... so we genuinely wanted to help newly diagnosed patients and other peers, share our treatment experience with them, and pass policy information on to

them so that they could avoid unnecessary difficulties” (D2). In a protected peer setting, his HIV experience was no longer a liability but became a basis for credibility. The central question is therefore not only whether information is useful or accessible, but whether a person has viable social and material conditions for acting on it and sustaining the resulting everyday arrangement.

4.4 Interweaving of the Three Practices

The three practices were analytically distinct but could become connected within or across situations. Within a situation, one action or arrangement could simultaneously reshape multiple practices. Across situations, connections emerged when the consequences of an earlier action or arrangement changed the conditions of later practice, such as information sources, audience boundaries, available support, or ways of acting. We treated situations as connected only when participants described such a concrete change, rather than temporal succession alone. Figure 1 summarizes these relationships and selected conditions through which they operated.

Alice’s medication arrangements illustrate within-situation interweaving. Clinical guidance became a regular dosing routine, but sustaining that routine around uninformed others required her to change the medication’s packaging and the setting in which she took it. Repackaging therefore did not simply conceal a cue, nor did adherence simply implement medical knowledge. The same arrangement redirected an identity-linked information flow and allowed her to follow treatment within everyday settings.

Her experience also shows how consequences carried forward. As treatment improved her indicators and physical condition, she increasingly relied on authoritative sources instead of searching broadly. The evidence does not establish clinical improvement as the sole cause of this change, but it shows that treatment experience became part of how she judged later information. At the same time, successful treatment continued to require medication and follow-up, so greater clinical confidence did not eliminate the need to navigate audiences. Different parts of the same trajectory could therefore move in different directions.

Jane’s trajectory began with a change in information flow rather than with a decision to seek or avoid content. Shared access to her phone expanded the potential audience for peer-group traces and led her to leave. When that arrangement changed, she rejoined but continued to participate selectively, directing specific questions to trusted people and using the group to offer help when peers needed medication. The phone thus shaped not only whether she could enter the information environment, but also how she could use it and contribute to it. Her case challenges a linear account in which engagement necessarily precedes information flow and action.

Supportive linkage produced another form of interweaving. Bella’s selective disclosure brought emotional support, but it did not make action equally possible across all areas of life; she still found it difficult to pursue intimacy despite her understanding of U=U. Richard’s trajectory moved further: care received in a trusted setting enabled him to become a source of treatment and policy information for others. These cases show that identity linkage can change a person’s position in an information environment, but its effects remain relationship- and domain-specific.

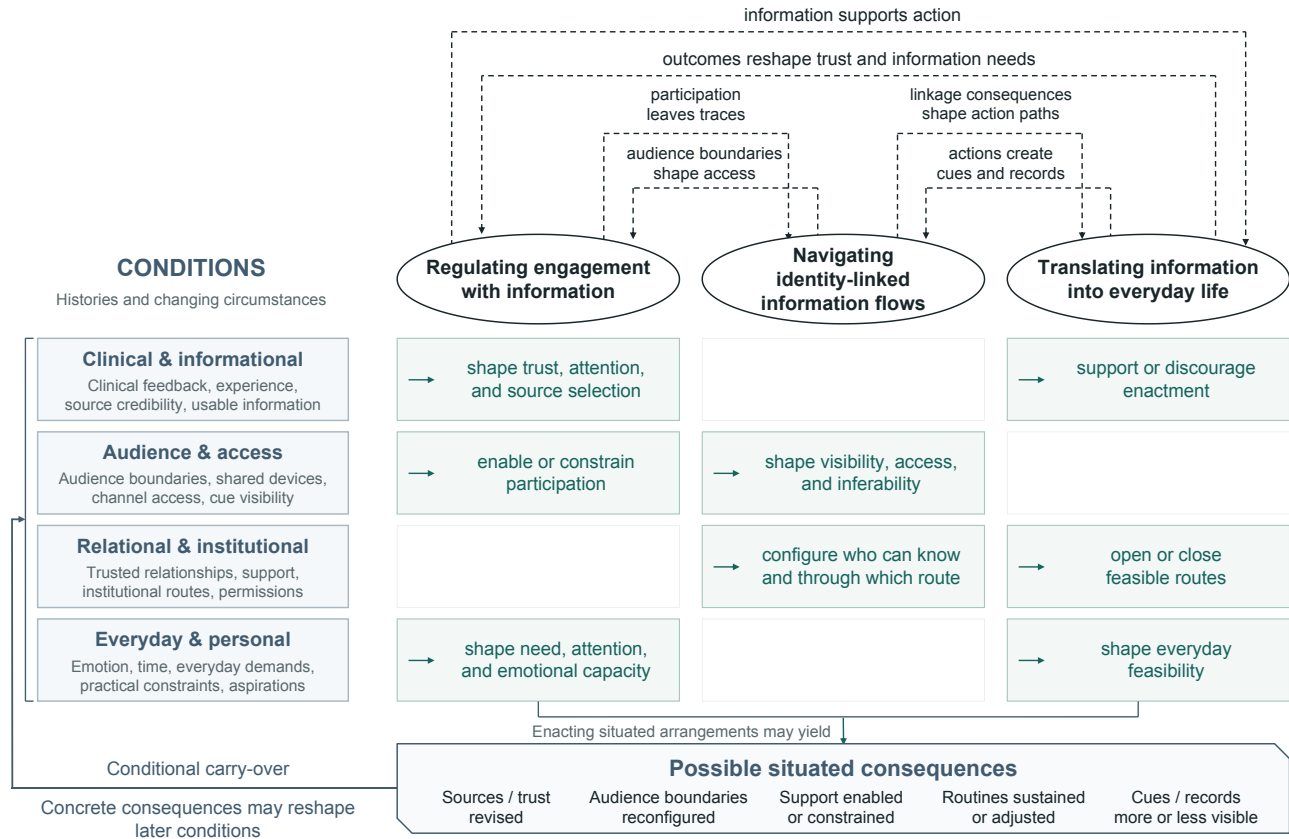


Figure 1: Conditions and connections among the three information practices. Matrix entries summarize selected ways in which conditions shape practices. Dashed arrows indicate possible connections.

Kate provides an important boundary case. Her reduced browsing followed from work and household demands, while clinically relevant lectures remained welcome. The evidence does not support a chain from an HIV-related cue to a particular audience in this instance. Her case therefore limits the reach of identity-linkage potential and shows why the interweaving of practices cannot be reduced to continuous privacy calculation. More generally, acting on information and incorporating it into daily life may change the conditions of later information engagement and action, but such a connection must be demonstrated through the concrete circumstances described by participants.

5 Discussion

5.1 Identity-Linkage Potential as a Cross-Practice Mechanism

Research has already shown that HIV disclosure can be selective, iterative, and involuntary, and that medication or digital records can make status visible [10, 42, 46, 60]. Identity-linkage potential shifts attention from whether disclosure has occurred to how recognizability becomes possible beforehand. A cue becomes identity-linked only in relation to an audience that can access it and interpret it as evidence of HIV status. Its significance also depends on the consequences associated with that inference and whether another route

is available. Linkage is therefore not an inherent property of a pill, record, device, or action.

This concept also clarifies the relationship between stigma and sociotechnical arrangements. Stigma explains why recognition may have consequences, whereas arrangements of access and interpretation explain how recognition becomes possible. This distinction matters when technical access and users' perceptions diverge. Restricted permissions may reduce actual exposure without reassuring users, whereas recognition within trusted relationships may enable support. Identity-linkage potential extends relational accounts of networked privacy [15, 45] by specifying one mechanism through which otherwise distinct information practices become connected.

5.2 From Information Access to Action

Research on health information has shown that information must be localized, appropriated, and made meaningful within everyday settings [25, 27, 64]. We extend this argument by separating access, understanding, and trust from whether and how people are able to act on information in everyday life. Bella's acceptance of U=U did not by itself make intimacy feasible. Nora's knowledge of legal and benefit procedures likewise did not provide a route she could comfortably use. In both cases, the barrier lay in the social and institutional arrangements required to act on it.

This distinction reframes apparent nonuse. A medically sound recommendation may be followed directly, carried out discreetly, acted on through another person or an alternative route, or not acted upon when no acceptable route is available. These outcomes should not be collapsed into a single measure of information use or automatically attributed to limited literacy. The analysis instead directs attention to the circumstances that shape whether and how people act, how they incorporate action into everyday routines and arrangements, and what consequences follow. Although developed from HIV-related experiences, this distinction may also help examine why information that is known, trusted, and desired is or is not acted upon in other contexts where treatment, identity, and institutional participation are closely connected.

5.3 Interwoven but Uneven Information Practices

The relationship among the three practices changes the unit of analysis from isolated information behaviors to evolving arrangements. The practices do not form a sequence from seeking to sharing and then using information. Within a situation, the same arrangement may simultaneously alter information engagement, identity-linked information flows, actual action, and everyday arrangements. Across situations, the consequences of an arrangement may alter the conditions under which information is later encountered or used. This extends feedback-oriented disclosure and transition research [4, 10] by showing that ordinary information-related actions, rather than only disclosure events or clearly bounded transitions, can change the conditions of later practice.

The process unfolds unevenly across different domains of life. Successful treatment may allow illness information to recede from attention, while supportive recognition may turn an information recipient into a trusted source for others. Linkage can close one domain of life while opening another. The relevant analytic unit is therefore the changing relationship among a person, an audience, and the arrangements through which information can or cannot be acted upon. Identity-linkage potential explains this process when a particular cue, access pathway, and possible inference can be identified. Clinical outcomes, practical constraints, and future aspirations may also reshape later practices independently of identity linkage.

5.4 Implications for Information Services

For digital libraries and related information services, our findings suggest that successful access cannot be assessed through relevance and accuracy alone. User studies should also examine whether people can engage with information without unwanted exposure and transform it into feasible action. Limited use may reflect emotional regulation, sufficient existing knowledge, or concern about access traces rather than low information literacy or disengagement from care. Evaluation should therefore distinguish difficulties in finding information from difficulties in using it under particular social and institutional conditions.

Privacy-sensitive service design requires attention to the interaction surrounding content. On shared devices, a notification or visible group membership may matter more than the information itself. In institutional systems, uncertainty about who can retrieve a record may discourage use even when technical permissions

are restricted. Information services can respond by making likely audiences and access rules more intelligible to users while supporting participation that does not require immediate public visibility. In peer settings, low-visibility access should remain connected to routes toward trusted human support, because complete anonymity may protect users while also limiting HIV-specific care.

These concerns also extend to AI-mediated health information. Prior work shows that large language models (LLMs) may produce health-condition-dependent judgments despite appearing non-stigmatizing on explicit measures [59]. Evaluation should therefore examine not only accuracy and respectful language, but whether disclosure of HIV status changes otherwise comparable responses or constrains users' viable routes to support and action.

6 Conclusion

This study shows that living with HIV involves not only finding and evaluating information but continually managing how information becomes linked to identity and made actionable in everyday life. We develop identity-linkage potential to explain how the accessibility and interpretability of ordinary cues can reshape information engagement, support, and action before disclosure or recognition occurs. The findings further show that accurate and trusted information may remain difficult to act upon when available routes create unacceptable identity-linkage consequences. Supporting equitable information access therefore requires attention not only to content, but also to the visibility of engagement and the availability of socially viable routes from information to action.

This study has several limitations. Fieldwork covered two months in one urban Chinese setting, and recruitment through a specialist hospital and peer-support organization may overrepresent PL-HIV already connected to treatment or community support. Most changes were reconstructed retrospectively rather than observed longitudinally, and participants' accounts may therefore be affected by recall. Researchers' volunteer roles may also have shaped field access, interaction, and interpretation. Some claims about institutional information access reflect participants' perceptions or a single staff account. Finally, identity-linkage potential was developed through qualitative interpretation in a specific context, and comparative and longitudinal research is needed to examine its operation across other populations and information environments.

References

- [1] Alanna J Bergman, Katherine C McNabb, Khaya Mlandu, Alvine Akumbom, and Dalmacio Dennis Flores. 2023. Identity management in the face of HIV and intersecting stigmas: A metasynthesis of qualitative reports from sub-Saharan Africa. *PLOS Global Public Health* 3, 2 (2023), e0000706. doi:10.1371/journal.pgph.0000706
- [2] Dale E. Brashers, Daena J. Goldsmith, and Elaine Hsieh. 2002. Information Seeking and Avoiding in Health Contexts. *Human Communication Research* 28, 2 (2002), 258–271. doi:10.1111/j.1468-2958.2002.tb00807.x
- [3] Lindsay K. Brown and Tiffany C. Veinot. 2021. Information Behavior and Social Control: Toward an Understanding of Conflictual Information Behavior in Families Managing Chronic Illness. *Journal of the Association for Information Science and Technology* 72, 1 (2021), 66–82. doi:10.1002/asi.24362
- [4] Lindsay K. Brown and Tiffany C. Veinot. 2026. Individual and Collective Transitions: Changes in Family Information Networks over Time in Life with Chronic Illness. *Journal of the Association for Information Science and Technology* 77, 2 (2026), 367–382. doi:10.1002/asi.24994
- [5] Michael Bury. 1982. Chronic Illness as Biographical Disruption. *Sociology of Health and Illness* 4, 2 (1982), 167–182.
- [6] Ne Cai, Dawei Wu, Kang Wang, and Honglin Jiang. 2026. An Information Behaviour Transition Model for the Self-Disclosure of HIV-positive Serostatus: A

- Qualitative Study. *Journal of Documentation* 82, 2 (2026), 427–452. doi:10.1108/JD-08-2025-0248
- [7] Bolin Cao, Suhui Dai, and Chang Liu. 2024. Old Friend, New Beginning: Redomesticating the Outdated ICTs for Biographical Reconstruction among PLH in China. *Information, Communication & Society* 27, 5 (2024), 915–934. doi:10.1080/1369118X.2023.2230258
- [8] Danièle Carricaburu and Janine Pierret. 1995. From Biographical Disruption to Biographical Reinforcement: The Case of HIV-positive Men. *Sociology of Health & Illness* 17, 1 (1995), 65–88. doi:10.1111/1467-9566.ep10934486
- [9] L. A. Chambers, S. Rueda, D. N. Baker, M. G. Wilson, R. Deutsch, E. Raefifar, S. B. Rourke, and The Stigma Review Team. 2015. Stigma, HIV and health: A qualitative synthesis. *BMC Public Health* 15, Article 848 (2015). doi:10.1186/s12889-015-2197-0
- [10] Stephenie R. Chaudoir, Jeffrey D. Fisher, and Jane M. Simoni. 2011. Understanding HIV Disclosure: A Review and Application of the Disclosure Processes Model. *Social Science & Medicine* 72, 10 (2011), 1618–1629. doi:10.1016/j.socscimed.2011.03.028
- [11] Annie T. Chen. 2022. Interactions between Affect, Cognition, and Information Behavior in the Context of Fibromyalgia. *Journal of the Association for Information Science and Technology* 73, 1 (2022), 31–44. doi:10.1002/asi.24538
- [12] Leesa Costello, Marie-Louise McDermott, and Ruth Wallace. 2017. Netnography: Range of practices, misperceptions, and missed opportunities. *International Journal of Qualitative Methods* 16 (2017), 1–12. doi:10.1177/1609406917700647
- [13] Constantinos K Coursaris and Ming Liu. 2009. An analysis of social support exchanges in online HIV/AIDS self-help groups. *Computers in Human Behavior* 25, 4 (2009), 911–918. doi:10.1016/j.chb.2009.03.006
- [14] Alexandra L. Dima, Sarah E. Stutterheim, Ramsey Lyimo, and Marijn De Bruin. 2014. Advancing methodology in the study of HIV status disclosure: The importance of considering disclosure target and intent. *Social science & medicine* 108 (2014), 166–174. doi:10.1016/j.socscimed.2014.02.045
- [15] Paul Dourish and Ken Anderson. 2006. Collective Information Practice: Exploring Privacy and Security as Social and Cultural Phenomena. *Human-Computer Interaction* 21, 3 (2006), 319–342. doi:10.1207/s15327051hci2103_2
- [16] G. H. Elder. 1998. The life course as developmental theory. *Child Development* 69, 1 (1998), 1–12. doi:10.1111/j.1467-8624.1998.tb06128.x
- [17] G. H. Elder, M. K. Johnson, and R. Crosnoe. 2003. The emergence and development of life course theory. In *Handbook of the Life Course*, Jeylan T. Mortimer and Michael J. Shanahan (Eds.). Springer US, Boston, MA, 3–19. doi:10.1007/978-0-306-48247-2_1
- [18] Christopher A. Faircloth, Craig Boylstein, Maude Rittman, Mary Ellen Young, and Jaber Gubrium. 2004. Sudden Illness and Biographical Flow in Narratives of Stroke Recovery. *Sociology of Health & Illness* 26, 2 (2004), 242–261. doi:10.1111/j.1467-9566.2004.00388.x
- [19] H. Fang. 2024. The moral (bio)politics of the abnormal: Situating a Southwest China border town in global AIDS governance. *Chinese Journal of Sociology* 10, 1 (2024), 131–163. doi:10.1177/2057150X241229033
- [20] Jennifer Fereday and Eimear Muir-Cochrane. 2006. Demonstrating Rigor Using Thematic Analysis: A Hybrid Approach of Inductive and Deductive Coding and Theme Development. *International Journal of Qualitative Methods* 5, 1 (2006), 80–92. doi:10.1177/160940690600500107
- [21] Michel Foucault. 2000. Lives of Infamous Men. In *Power: Essential Works of Foucault, 1954–1984*, James D. Faubion (Ed.). Vol. 3. The New Press, New York, NY, 157–175. Translated by Robert Hurley.
- [22] Nicola K. Gale, Gemma Heath, Elaine Cameron, Sabina Rashid, and Sabi Redwood. 2013. Using the Framework Method for the Analysis of Qualitative Data in Multi-Disciplinary Health Research. *BMC Medical Research Methodology* 13 (2013), 117. doi:10.1186/1471-2288-13-117
- [23] Alison Hicks, Pamela McKenzie, Jenny Bronstein, Jette Seiden Hyldegård, Ian Ruthven, and Gunilla Widén. 2025. Information Avoidance: A Critical Conceptual Review. An Annual Review of Information Science and Technology (ARIST) Paper. *Journal of the Association for Information Science and Technology* 76, 1 (2025), 326–346. doi:10.1002/asi.24968
- [24] Timothy P. Hogan and Carole L. Palmer. 2005. “Information Work” and Chronic Illness: Interpreting Results from a Nationwide Survey of People Living with HIV/AIDS. *Proceedings of the American Society for Information Science and Technology* 42, 1 (2005). doi:10.1002/meet.14504201150
- [25] Isto Huvila. 2015. Situational Appropriation of Information. *Aslib Journal of Information Management* 67, 5 (2015), 492–504. doi:10.1108/AJIM-02-2015-0033
- [26] Larissa Jennings Mayo-Wilson, Bianca Devoto, Jessica Coleman, Barbara Mukasa, Angela Shelton, Sarah MacCarthy, Uzaib Saya, Harriet Chemusto, and Sebastian Linnemayr. 2020. Habit formation in support of antiretroviral medication adherence in clinic-enrolled HIV-infected adults: a qualitative assessment using free-listing and unstructured interviewing in Kampala, Uganda. *AIDS research and therapy* 17, 1 (2020), 30. doi:10.1186/s12981-020-00283-2
- [27] Elizabeth Kazianas, Mark S. Ackerman, and Tiffany C. E. Veinot. 2013. Localizing Chronic Disease Management: Information Work and Health Translations. *Proceedings of the American Society for Information Science and Technology* 50 (2013), 1–10. doi:10.1002/meet.14505001090
- [28] E. N. Kontomanolis, S. Michalopoulos, G. Gkasdaris, and Z. Fasoulakis. 2017. The social stigma of HIV/AIDS: Society’s role. *HIV/AIDS - Research and Palliative Care* 9 (2017), 111–118. doi:10.2147/HIV.S129992
- [29] R. V. Kozinets. 2002. The field behind the screen: Using netnography for marketing research in online communities. *Journal of Marketing Research* 39, 1 (2002), 61–72. doi:10.1509/jmkr.39.1.61.18935
- [30] A. Langley. 1999. Strategies for theorizing from process data. *Academy of Management Review* 24, 4 (1999), 691–710. doi:10.5465/amr.1999.2553248
- [31] J.-Y. Li, S. Qiao, S. Harrison, and X. Li. 2017. Utilizing an interpersonal communication framework to understand information behaviors involved in HIV disclosure. *International Journal of Information Management* 37, 4 (2017), 250–256. doi:10.1016/j.ijinfomgt.2016.12.001
- [32] Z. Li, J. P. Morano, K. Khoshnood, E. Hsieh, and Y. Sheng. 2018. HIV-related stigma among people living with HIV/AIDS in rural Central China. *BMC Health Services Research* 18, 1, Article 453 (2018). doi:10.1186/s12913-018-3245-0
- [33] Calvin Liang, Jevan Alexander Hutson, and Os Keyes. 2020. Surveillance, Stigma & Sociotechnical Design for HIV. *First Monday* 25, 10 (2020). doi:10.5210/fm.v25i10.10274
- [34] Teri G. Lindgren, Darcel Reyes, Lucille Eller, Dean Wantland, Carmen Portillo, William L. Holzemer, Ellah Matshediso, Inge Corless, Jeanne Kempainen, K. Dintle Mogobe, Allison Webel, Kathleen Nokes, Patrice Nicholas, Marta Rivero Mendez, Paula Reid, Solymar Solis Baez, Mallory O. Johnson, Yvette Cuca, and Carol Dawson Rose. 2018. Understanding Health Literacy for People Living with HIV: Locations of Learning. *Journal of the Association of Nurses in AIDS Care* 29, 2 (2018), 190–203. doi:10.1016/j.jana.2017.10.007
- [35] Annemaree Lloyd. 2010. Corporeality and practice theory: exploring emerging research agendas for information literacy. *Information Research* 15, 3 (2010), 15–3.
- [36] Annemaree Lloyd. 2012. Information literacy as a socially enacted practice: Sensitising themes for an emerging perspective of people-in-practice. *Journal of documentation* 68, 6 (2012), 772–783. doi:10.1108/00220411211277037
- [37] Annemaree Lloyd, Ann Bonner, and Carol Dawson-Rose. 2014. The Health Information Practices of People Living with Chronic Health Conditions: Implications for Health Literacy. *Journal of Librarianship and Information Science* 46, 3 (2014), 207–216. doi:10.1177/0961000613486825
- [38] Annemaree Lloyd-Zantiotis and M. Olsson. 2017. Being in place: embodied information practices. *Information Research: an international electronic journal* 22, 1 (2017).
- [39] Fadhila Mazanderani and Ian Brown. 2011. Privacy as a Practice: Exploring the Relational and Spatial Dynamics of HIV-Related Information Seeking. In *Computers, Privacy and Data Protection: an Element of Choice*, Serge Gutwirth, Yves Poulet, Paul De Hert, and Ronald Leenes (Eds.). Springer Netherlands, Dordrecht, 251–268. doi:10.1007/978-94-007-0641-5_12
- [40] Fadhila Mazanderani, Nicholas Hughes, Claire Hardy, Elizabeth Sillence, and John Powell. 2019. Health information work and the enactment of care in couples and families affected by Multiple Sclerosis. *Sociology of Health & Illness* 41, 2 (2019), 395–410. doi:10.1111/1467-9566.12842
- [41] Pamela J. McKenzie. 2003. A model of information practices in accounts of everyday-life information seeking. *Journal of documentation* 59, 1 (2003), 19–40. doi:10.1108/00220410310457993
- [42] Charles Muiruri, Shelley A. Jazowski, Seleman K. Semvua, Francis P. Karia, Brandon A. Knettel, Leah L. Zullig, Habib O. Ramadhani, Blandina T. Mmbaga, John A. Bartlett, and Hayden B. Bosworth. 2020. Does Antiretroviral Therapy Packaging Matter? Perceptions and Preferences of Antiretroviral Therapy Packaging for People Living with HIV in Northern Tanzania. *Patient Preference and Adherence* 14 (2020), 153–161. doi:10.2147/PPA.S238759
- [43] Helen Nissenbaum. 2004. Privacy as Contextual Integrity. *Washington Law Review* 79 (2004), 119–157.
- [44] Carla Makhlof Obermeyer, Parijat Bajjal, and Elisabetta Pegurri. 2011. Facilitating HIV disclosure across diverse settings: a review. *American journal of public health* 101, 6 (2011), 1011–1023. doi:10.2105/AJPH.2010.300102
- [45] Leysia Palen and Paul Dourish. 2003. Unpacking “Privacy” for a Networked World. In *Proceedings of the SIGCHI Conference on Human Factors in Computing Systems*. Association for Computing Machinery, New York, NY, USA, 129–136. doi:10.1145/642611.642635
- [46] Chrysanthi Papoutsis and Ian Brown. 2015. Privacy as Articulation Work in HIV Health Services. In *Proceedings of the 18th ACM Conference on Computer Supported Cooperative Work & Social Computing*. Association for Computing Machinery, New York, NY, USA, 339–348. doi:10.1145/2675133.2675204
- [47] R. Parker and P. Aggleton. 2003. HIV and AIDS-related stigma and discrimination: A conceptual framework and implications for action. *Social Science & Medicine* 57, 1 (2003), 13–24. doi:10.1016/S0277-9536(02)00304-0
- [48] Lance S Rintamaki, Terry C Davis, Silvia Skripkauskas, Charles L Bennett, and Michael S Wolf. 2006. Social stigma concerns and HIV medication adherence. *AIDS patient care and STDs* 20, 5 (2006), 359–368. doi:10.1089/apc.2006.20.359
- [49] Ian Ruthven. 2022. An Information Behavior Theory of Transitions. *Journal of the Association for Information Science and Technology* 73, 4 (2022), 579–593. doi:10.1002/asi.24588

- [50] GW Ryan and GJ Wagner. 2003. Pill taking ‘routinization’: a critical factor to understanding episodic medication adherence. *AIDS care* 15, 6 (2003), 795–806. doi:10.1080/09540120310001618649
- [51] Parya Saber, Megan Comfort, and Mallory O Johnson. 2012. The relationship between daily organization and adherence to antiretroviral therapy: a qualitative perspective. *Journal of Acquired Immune Deficiency Syndromes* 61, 1 (2012), e3–e6. doi:10.1097/QAI.0b013e318261faf2
- [52] Reijo Savolainen. 2007. Information Behavior and Information Practice: Reviewing the “Umbrella Concepts” of Information-Seeking Studies. *The Library Quarterly* 77, 2 (2007), 109–132. doi:10.1086/517840
- [53] Sarah E Stutterheim, Arjan ER Bos, John B Pryor, Ronald Brands, Maartje Liebrechts, and Herman P Schaalma. 2011. Psychological and social correlates of HIV status disclosure: The significance of stigma visibility. *AIDS Education and prevention* 23, 4 (2011), 382–392. doi:10.1521/aeap.2011.23.4.382
- [54] Megan Threats. 2025. Toward Information Resilience: Applying Intersectionality to the HIV AIDS Information Practices of Black Sexual Minority Men. *Journal of the Association for Information Science and Technology* 76, 8 (2025), 1123–1140. doi:10.1002/asi.24999
- [55] Kimmo Tuominen, Sanna Talja, and Reijo Savolainen. 2005. The social constructionist viewpoint on information practices. In *Theories of information behavior*. Information Today, United States, 328–333.
- [56] Christine Vaughan, Glenn Wagner, Lisa Miyashiro, Gery Ryan, and James D Scott. 2011. The role of the home environment and routinization in ART adherence. *Journal of the International Association of Physicians in AIDS Care* 10, 3 (2011), 176–182. doi:10.1177/1545109711399365
- [57] Tiffany Veinot. 2009. “A Lot of People Didn’t Have a Chance to Support Us Because We Never Told Them” Stigma Management, Information Poverty and HIV/AIDS Information/Help Networks. *Proceedings of the American Society for Information Science and Technology* 46, 1 (2009), 1–20. doi:10.1002/meet.2009.1450460273
- [58] T. Veinot. 2009. Interactive acquisition and sharing: Understanding the dynamics of HIV/AIDS information networks. *Journal of the American Society for Information Science and Technology* 60, 11 (2009), 2313–2332. doi:10.1002/asi.21151
- [59] Xi Wang, Yujia Zhou, and Guangyu Zhou. 2026. Large Language Models Exhibit Stigmatizing Behaviour in Contextual Judgements of Health Conditions. *Nature Health* (2026). doi:10.1038/s44360-026-00164-4
- [60] Mark Warner, Andreas Gutmann, M. Angela Sasse, and Ann Blandford. 2018. Privacy Unraveling around Explicit HIV Status Disclosure Fields in the Online Geosocial Hookup App Grindr. *Proceedings of the ACM on Human-Computer Interaction* 2, CSCW, Article 181 (2018), 22 pages. doi:10.1145/3274450
- [61] Rebekah Willson. 2019. Transitions Theory and Liminality in Information Behaviour Research: Applying New Theories to Examine the Transition to Early Career Academic. *Journal of Documentation* 75, 4 (2019), 838–856. doi:10.1108/JD-12-2018-0207
- [62] Thomas D Wilson. 1999. Models in information behaviour research. *Journal of documentation* 55, 3 (1999), 249–270. doi:10.1108/EUM000000007145
- [63] Thomas D Wilson. 2000. Human information behavior. *Informing science* 3, 2 (2000), 49–55.
- [64] Christine T. Wolf and Tiffany C. Veinot. 2015. Struggling for Space and Finding My Place: An Interactionist Perspective on Everyday Use of Biomedical Information. *Journal of the Association for Information Science and Technology* 66, 2 (2015), 282–296. doi:10.1002/asi.23178
- [65] L. H. Yang and A. Kleinman. 2008. ‘Face’ and the embodiment of stigma in China: The cases of schizophrenia and AIDS. *Social Science & Medicine* 67, 3 (2008), 398–408. doi:10.1016/j.socscimed.2008.03.011
- [66] Y. C. Zhao, D. Wu, and S. Song. 2026. Meaning-making during mental health struggles: Transitional information practices among individuals with obsessive-compulsive disorder. *Journal of the Association for Information Science and Technology* 77(2) (2026), 347–366. doi:10.1002/asi.25003
- [67] Y. R. Zhou. 2007. “If you get AIDS... You have to endure it alone”: Understanding the social constructions of HIV/AIDS in China. *Social Science & Medicine* 65, 2 (2007), 284–295. doi:10.1016/j.socscimed.2007.03.031